

Ear - Swimmer's - Female (Child)

Algorithm 1164 | Child (0-17 years) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric pathway for suspected otitis externa after water exposure, screening for invasive infection, diabetes/immunocompromise, hearing or neurologic complications; not approved for production.
- Child patient.
- Female.

Initial assessment

1. [DURATION] How long have symptoms lasted?
2. [YES_NO] Recent swimming or water exposure?
3. [YES_NO] Any discharge, hearing change, or dizziness?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant; and are there severe/night pain, fever, swelling beyond the canal, facial weakness, diabetes, immune suppression, recent instrumentation or safeguarding concerns?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, fluid intake, urine output, pain, fever, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Earache

Is there ear pain without recent swimming, water exposure, or outer-ear canal inflammation?

Ear pain not associated with water exposure or an inflamed ear canal belongs in the general Earache pathway.

1. Disposition level 100

Is there confusion, reduced responsiveness, rapid deterioration, facial weakness, severe pain with diabetes or immune suppression, or redness/swelling spreading around or behind the ear with marked systemic illness?

These features may indicate sepsis, invasive external-ear infection or cranial-nerve involvement and require emergency assessment.

1. Disposition level 70

Does the person feel generally unwell or have a very high temperature, is there swelling around the ear, fluid coming from the ear, hearing loss or change, nausea/vomiting/dizziness, a severe sore throat, or a weakened immune system?

NHS.UK ear infection guidance lists these as urgent clinical-review criteria.

1. Disposition level 50

Has earache persisted beyond 3 days, or are ear infections recurring?

NHS.UK guidance recommends a primary-care review for persistent or recurrent ear infections.

1. Disposition level 70

After higher-risk features are excluded, is swimmer's ear still suspected without direct canal and eardrum examination?

The cited source is not specific to otitis externa and safe drops depend on eardrum status; this UAT pathway requires in-person examination.

Advice

Emergency invasive ear-infection precautions

- Call Qatar 999 and do not drive. Keep the patient observed and the ear dry; insert nothing and use no drops while awaiting emergency assessment. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: confusion or reduced responsiveness; facial weakness or rapidly spreading swelling.

Urgent ear infection review

- Arrange same-day medical review for these symptoms.
- Call Back If: worsening pain or swelling; fever develops.

Routine ear infection follow-up

- Arrange a primary-care review for persistent or recurring ear infections.
- Call Back If: symptoms worsen; new discharge.

In-person assessment for suspected swimmer's ear

- Keep the ear dry, avoid swimming and do not insert objects or use drops until the canal and eardrum are examined. Arrange prompt in-person assessment through the Qatar UAT pathway.
- Call Back If: pain or swelling worsens; fever, hearing change, discharge, dizziness or facial weakness.

First aid

- Call Qatar 999 and do not drive. Keep the patient observed and the ear dry; insert nothing and use no drops while awaiting emergency assessment. Call Qatar 999 now, follow the call-taker's ambulance and

emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Ear infection", <https://www.nhs.uk/conditions/ear-infection/> (page last reviewed 16 January 2025)

Provenance

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